

SECTION VI

Associated Features of Trauma

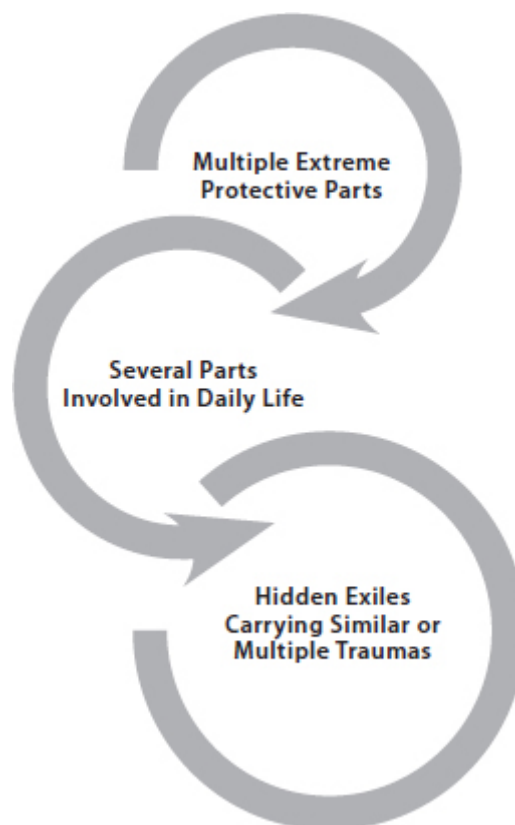
CHAPTER 18

Systems, Severe Trauma, and Polarities

IDENTIFYING COMPLICATED SYSTEMS

The constellation of parts (i.e., systems formed around repeated relational trauma) tends to be complicated, confusing, and often undiscovered for quite some time. It's even more pronounced and perplexing with clients who suffer from dissociative disorders. However, consistent patterns tend to emerge within these complex systems if you're open to discovering them and allow them to unfold accordingly.

For example, there is often a *series of extreme parts*, including suicidal, destructive, critical, cutting, and substance-using parts. There are also several *very strong managers* that can run the client's day-to-day life. Deep inside, often out of the client's conscious awareness, lie the *hidden exiles*. These exiles can be singular, such as a "little Jane" who carries multiple traumas over time, including being yelled at by her mom, violated by boys in the neighborhood, and bullied at school.



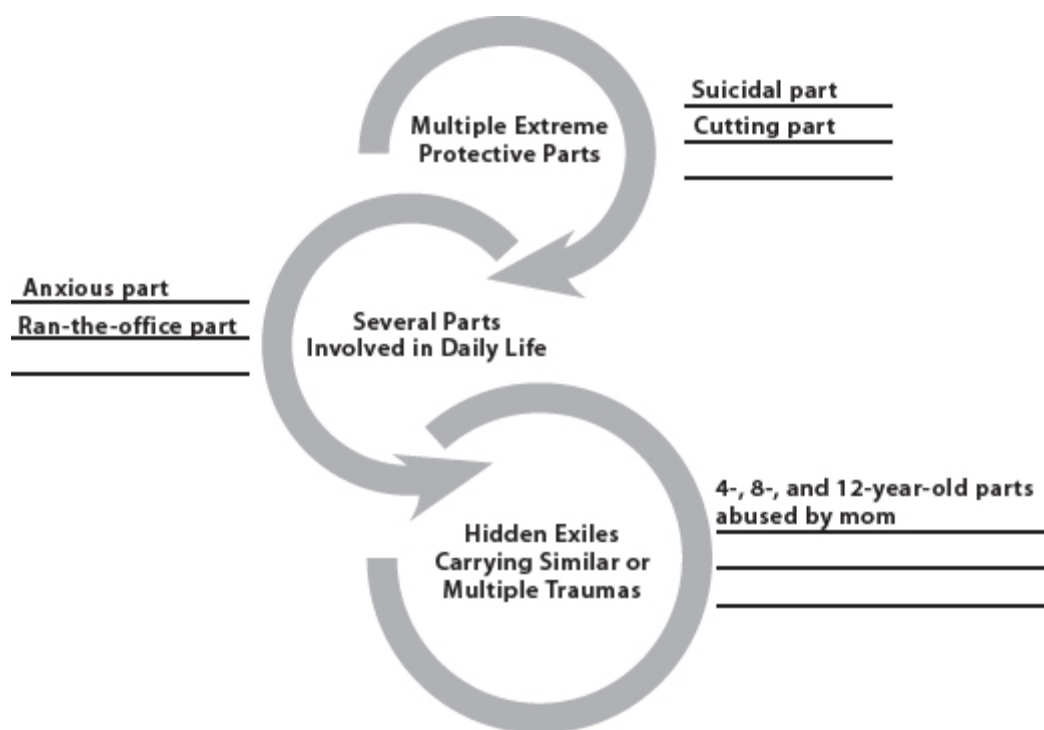
There can also be multiple exiles that carry similar trauma over time, such as being sexually abused by a brother at 4, 10, and 13 years old. Or there can be multiple exiles holding multiple traumas that are organized within an elaborate, interconnected system (e.g., a 4-year-old who witnessed parental fighting, an 8-year-old traumatized by sibling bullying, and a 13-year-old holding on to abuse by an uncle).

The diagram here shows how these complex systems interrelate.

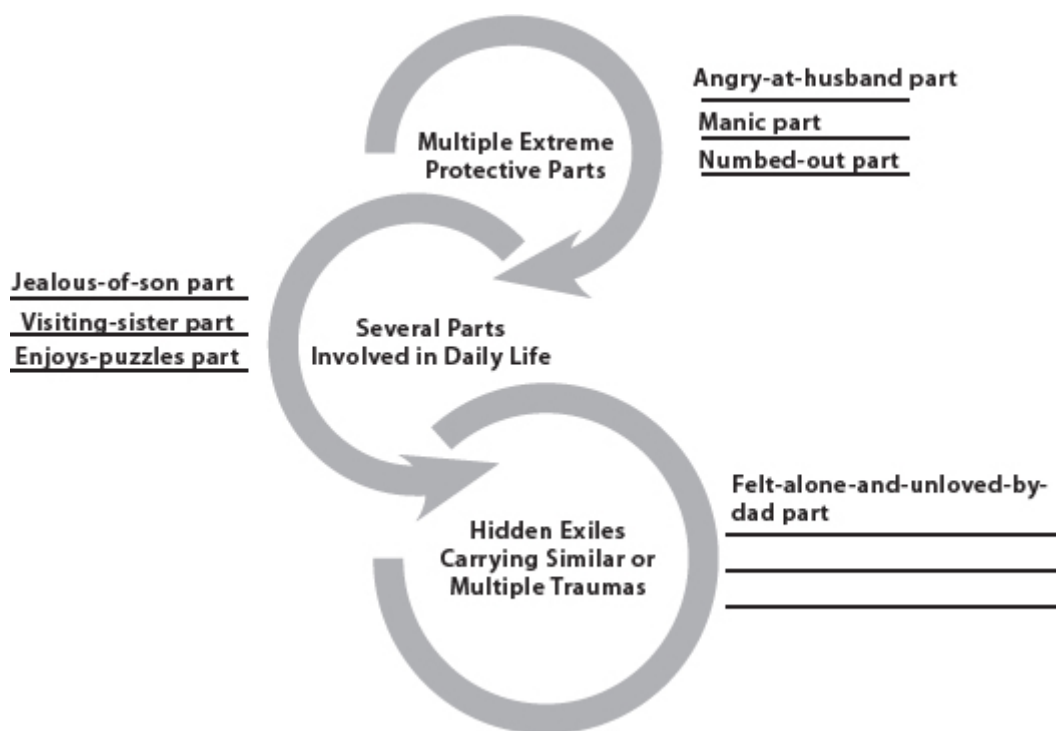
Consider the following case example. I worked with my client, Martha, for several years on her issues with DID. We spent most of our time getting to know the numerous parts of her that dominated Martha's life. There was a suicidal part that didn't care whether she lived or died, a cutting part that only cut in hidden areas of her body, a part that went to work and managed over 50 employees in her department, a part that hated her husband, a part that went to visit her verbally abusive sister weekly, a part that was raped in college, a part that liked to do puzzles, a part that felt numb, a part that ran the house and managed the kids, a part that became manic, a part that was jealous of her son's success, and an anxious part that jumped in every time Martha felt overwhelmed. And this was only a small sample of Martha's many parts.

Most sessions were spent interviewing these parts, either through insight or direct access. Slowly, over time, I began to notice that certain parts were consistently mentioned together. They started coalescing into groups. For example, the part that ran the house, the anxious part, the cutting part, and the suicidal part would often show up together. Meanwhile, the parts that were angry at Martha's husband, were jealous of her son, got manic, numbed her out, visited her sister, and enjoyed completing puzzles often came together too.

As I started sharing these groupings with Martha, she was well aware of the fact that they were connected but was never forthcoming with this information. She also was reluctant to share whom these groups were protecting. "Sharing that information would make me vulnerable, and I don't do vulnerable," Martha told me. I didn't push back, but I always held this information about Martha's different systems in the back of my mind. It felt like I was putting together a giant jigsaw puzzle mostly by myself. Every once in a while, I would get another piece of the puzzle when we interviewed one of her parts. As shown below, after working together for about two-and-a-half years, we learned that the anxious, cutting, suicidal, and hard-working parts were all protecting three little exiles: those stuck in ages 4, 8, and 12. These parts held on to the sexually inappropriate things Martha's mother would do to her when she was drunk.



We also learned that the parts that were angry at her husband, jealous of her son, manic, numbed out, enjoyed puzzles, and visited her sister were protecting a little girl who felt alone and unloved by her father.

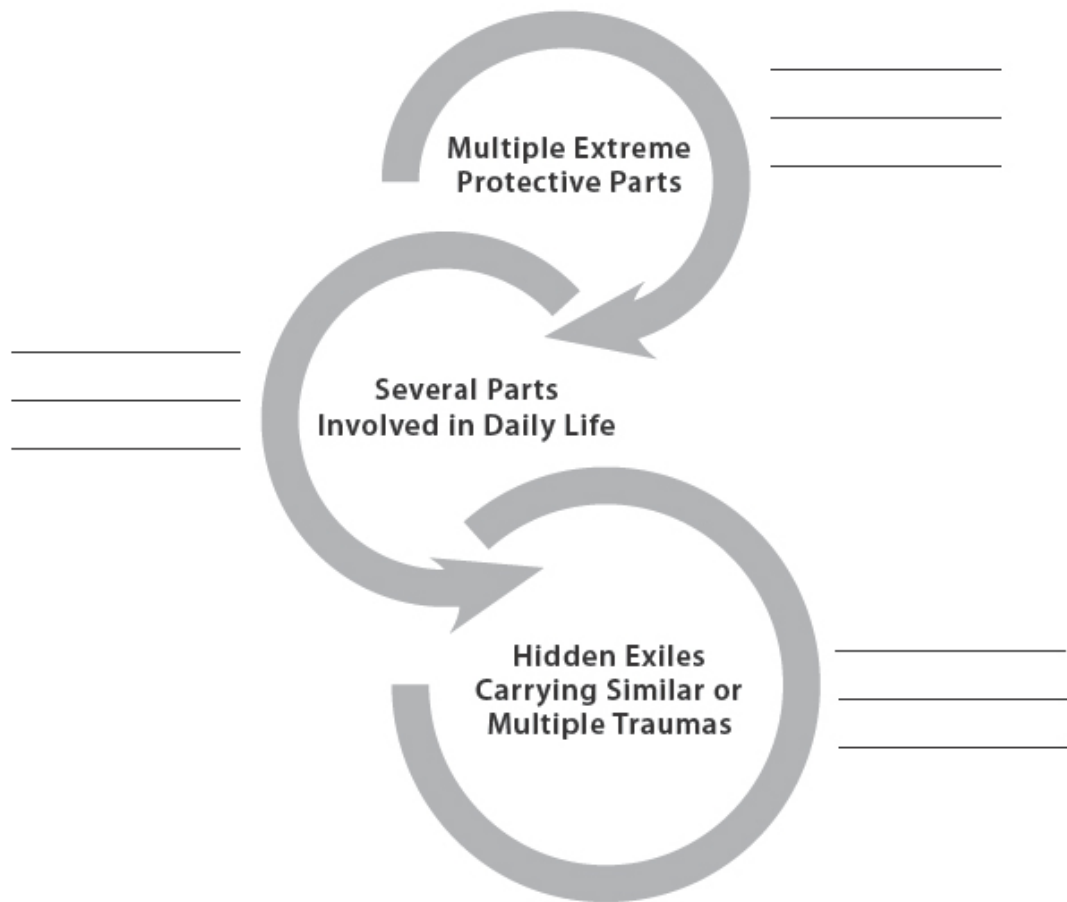


With dissociative disorders, protectors are often part of an elaborate system, reluctant to separate, slow to trust, and hesitant to reveal whom they’re protecting. It can take months to years before you and your clients get a sense of who is protecting what. In the early stages of treatment, it’s common to engage in *implicit direct access* with the myriad of protectors that show up in your clients. Here, we

talk in general to them, attempt to get to know them, and begin to see how they relate to one another and interact within the system as a whole.

Be mindful that some protective parts in dissociative systems are reluctant to be seen and known, so calling them out too soon can be problematic. However, over time, as they begin to feel safe and more at ease, switching to *explicit direct access* is often an essential strategy in getting to know their job, learning about their fears, and gaining parts' permission to access the wound they're protecting. You may also need to offer *the invitations* (as discussed in [chapter 9](#)) hundreds of times before they are truly willing to allow the Self to be in connection with the exile. Remember, it can be a single exile holding many traumas, several exiles holding similar painful events, or several exiles holding different but related experiences. Being patient is the key to solving and, ultimately, healing these complex puzzles.

Here is a blank template you can use to map out complicated systems within your clients.



JOINING THE SYSTEM

As we can see, systems that develop as a result of trauma can be intricate, elaborate, and sometimes convoluted. When we see clients over a long span, it's easy to inadvertently slip or get indoctrinated

into their belief system. With complex trauma, there's often a desperation or futility that ensues within the system, especially after many parts have been trying to solve a problem unsuccessfully for years, which they realize. They're generally not open to new ideas and tend to slowly convince therapists about what happened and what will happen.

Make no mistake: Therapists can get brainwashed by parts and begin to buy into their beliefs. You may find yourself thinking: "Maybe nothing will ever significantly change for Samantha," "Perhaps Ava doesn't have enough Self-energy to ever fully recover from her rape," "It's possible Theo is better off living alone," or "Maybe IFS doesn't work for this client."

Consider the case of Jody, a long-term client of mine who had a horrible trauma history that included a schizophrenic mother and an alcoholic father who worked as a traveling salesperson. Jody's early attachment history was anything but secure. She worked hard in therapy to try to have healthy adult relationships, despite her upbringing. I would have probably diagnosed Jody with dissociative disorder not otherwise specified (DDNOS) before my IFS training. She didn't have distinct shifts from one part to another but would subtly and almost seamlessly move in and out of parts without much notice.

Jody would frequently get discouraged and blame me for her lack of progress in therapy as well as in her life. "This therapy isn't helping me," I would often hear her say. "I've spent so much money here over the years, and what do I have to show for it? Nothing. You're supposedly so well-known. You should be good at this stuff. Why aren't you helping me get better? I need someone who cares about me as a person, not someone who travels all the time and writes books."

After one of these episodes, I would feel inadequate and alternate between thinking that Jody either lacked the capacity to heal or that I lacked the skills she needed to guide that healing. My supervision group helped me see that I was getting indoctrinated into Jody's system of hopelessness. I began to examine my parts that were believing Jody's parts. What was striking was that I wasn't feeling these emotions with any other clients or in any other areas of my life.

I became aware of a part of me that didn't like being blamed for Jody's struggles and another younger part that would constantly try to fix her problems. It was a relief to be able to identify these parts within me and appreciate the ways they slowly became integrated into Jody's system. I was able to unblend from these parts and reject responsibility for her complaints about me. As a result, our work shifted pretty dramatically. I was able to embody more objectivity and compassion while Jody began to understand and own her feelings more fully. She was even able to identify a thought-

disordered part that internalized her mother's projections and distortions of her as a child. Jody ultimately decided to take a low-dose antipsychotic medication, which made a huge difference in our work together and in her ability to connect with others with less suspicion and more vulnerability.

We often inevitably and dangerously get drawn into a client's system. This is a result of our parts being subconsciously willing to join in a belief that the client's parts subliminally suggest. The challenge here is to remain within the IFS framework, stay the course, and not buy into the opinion or stance of the client's system. *We need to feel confident enough to challenge these beliefs, extricate ourselves from them, unblend from our activated parts, and convince the client's parts that there's another way to see this and resolve this problem.* Using the right language can help immensely: "I know you're feeling hopeless right now, but my perspective is that you can get beyond what happened to you when you were younger. I know you feel incapable of feeling any love in your life right now, but I know it's possible for you. If your parts permit us, we can heal the part of you that believes you are not capable of loving someone."

These hopeless parts tend to block progress from the inside out. They are firmly entrenched in their beliefs and work hard to convince us that their story is true. Once we remove ourselves from the client's system, we can gain firsthand experience of the nature of their pain and the difficulty and hopelessness the system feels. We can then compassionately and effectively rise above it, challenge the story, and separate ourselves from the ubiquitous nature of their experience. When we get drawn into our client's pessimistic stories, it's important to monitor our parts and trust in the healing capacity of Self-energy and the effectiveness of the IFS model. Always remember: We can help our clients heal, even if their tenacious belief system is convinced that it's not possible.

At times, it's helpful to join their system and experience what they feel firsthand. At times, it's necessary to step back, challenge any misleading and malformed beliefs, and suggest how they can unload the hurt and free themselves from carrying the burden of their experiences.

I can now see how my own trauma history and misguided parts have caused me to get sucked into my clients' systems at times—either by carrying too much of their pain, trying too hard to understand or figure things out without seeing the big picture, or tolerating too much and not setting appropriate limits.

DISSOCIATIVE IDENTITY DISORDER

Clients with more severe abuse histories that include ongoing sexual, physical, and verbal abuse seem to have a wide range of parts, both extreme and preventive, that dominate most of their sleeping and waking hours. These parts have a difficult time relaxing enough for Self-energy to emerge. This has

been an ongoing source of frustration and confusion for me over the years with many of my clients who struggle with DID.

I have often heard Dick Schwartz say that when parts step back, the Self will naturally emerge. In my heart, I know this to be true. But with many of my clients who have DID, this is not always the case. Some clients seem to have an extremely hard time accessing Self-energy. It's easy for me to become frustrated with myself and wonder: "What am I doing wrong? Why can't I help my client achieve what Dick says to be true?" At other times, parts of me get frustrated with my clients and their seeming lack of capacity to be able to access Self-energy. I find myself wondering: "This must be one of the exceptions to Dick's rule. Is the problem me and my lack of skill, or is it the client and their lack of capacity?"

For clients with DID, in particular, it can feel like a "parts festival" as clients constantly shift from one extreme part to another. And if by chance a Self-like manager shows up, it's easy to get fooled into thinking that the Self is finally present. Hence, you follow the model's steps only to become disappointed when things don't proceed as you know they should. It's quite common for clients who struggle with dissociative disorders to have *highly sophisticated Self-like parts*. They've needed to develop parts that can show up and interact in the world. These parts have carefully observed people and figured out how to fit into society. They're very convincing and often trick therapists into believing they are the Self.

One of the ways to determine if the Self or a Self-like part is present is to ask your client directly: "Is the compassion coming from your head or your heart?" This will help the client differentiate between the two and enable them to begin to sense what the Self feels like internally. Over time, you will get more proficient at identifying the openness and will sense the softening and body relaxation that accompanies Self-energy in your clients. You'll also become more adept at detecting Self-energy in others once you've developed a comfort knowing what it feels like within yourself.

Hallmark symptoms of DID, as they relate to IFS, include the *number, frequency, and intensity of polarized parts* present at any one time. It's not only a parts festival—it's also a polarization bonanza. One part is conflicted with another part that's in opposition with a different part and so on. Sometimes I'm convinced that my clients with DID leave my office and say silently to themselves: "Yes, we've just confused the hell out of him again. Mission accomplished." If protective parts can keep us confused, frightened, and overwhelmed by the intensity, severity, impulsivity, and complexity of their presentation, they feel successful yet again at keeping the vulnerability at bay. It's a brilliant way to stay away from trauma feelings and memories if you stop and think about it.

Clients with DID will also have several parts that *collude with each other*, form packs, and present like a well-orchestrated magic show. They continually lie, hide, and distort the truth, and they are often reluctant to reveal anything about their colluding compatriots. For proof, ponder the following conversation I once had with a client's teenage part:

"Do you know about the part that calls itself 'fear baby'?" I asked.

"Yes, of course I do," it replied.

"Well, that's news to me," I said, trying to keep my frustrated parts at bay. "Is anyone else in there hiding or lying right now?"

A 9-year-old part spoke up and said: "I've been hiding for a while, and I lied when you asked if we were ever bullied by my sister."

It's important to not take the lying and withholding personally; remember this behavior is in the service of protection.

Working with clients who have DID can often feel like a roller-coaster ride. You experience a series of ups and downs with extreme hyperaroused and numbing dissociative parts that switch back and forth at what feels like the speed of light. You feel like you're constantly *chasing one crisis after another*, never fully having a sense as to what is going on inside of your client's mind. I've come to understand that this is the point: Protective parts want to keep you and your client confused and guessing so exiled parts remain out of conscious awareness and unexposed in any way.

Parts can initially *present symbolically* or in an array of nonhuman forms—for example, as a fire-breathing dragon, a dark smoky figure, a cackling witch, fog, a boulder, a locked safe, or a brick wall. As the relationship develops between a part and the Self, they typically, but not always, transform into human form. Staying open, without judgment or agenda, will allow the part to evolve as it sees fit. You may also notice that clients with DID often display *rapid eye movements* when they're thinking about something or retreating inward. I often see this and wonder what's going on inside their head, knowing that a very small percentage ever gets spoken aloud to me.

Therapists need to learn, mostly through experience, to ride out the waves in a nonreactive, matter-of-fact manner so that the client's system realizes you won't get flustered by all the drama. Protectors will naturally begin to relax and trust you as their tour guide through the healing journey. Remember that clients who struggle with DID often have parts that *hate the Self* and have no interest in getting to know it. They'll need to trust you before they're willing to trust the one who, in their eyes, betrayed

them and left them behind to endure the torture. *The Self can initially appear weak and marginally accessible* in clients who struggle with DID, but over time it becomes more consistent, dependable, and readily available as parts get to know it better and develop a secure relationship with it.

Know that IFS therapy with clients who have DID is essentially the same as it is with other trauma survivors, but it tends to be more challenging, activate more parts of the therapist, and involve more extreme symptoms, and it takes much longer to help clients access Self-energy, get permission from protectors, and heal or unburden wounds.

Clients with DID

- **Numerous parts, many of them extreme**
- **Highly evolved Self-like parts**
- **Multiple polarizations**
- **Parts that collude and form alliances**
- **Parts that hide, exaggerate, or are dishonest**
- **Crisis-driven systems**
- **Parts that present symbolically**
- **Rapid eye movements are common**
- **Parts often hate the Self**
- **The Self is marginally accessible**

THE INVISIBLE INJURIES OF NEGLECT

Neglect, in some way, can be as challenging as DID. These systems tend to be steadfast, rock-solid, and unwavering in their attempt to keep the lack of love and connection permanently hidden. There's a desperation to fill the void with something that keeps the feeling of isolation away. As previously discussed, protective parts associated with neglect injuries often show up in the form of thinking, analyzing, figuring out, and perfectionism. They're commonly part of a shame cycle. They can be pervasive within the system and tenacious in their reluctance to step back or separate, thinking: "Why would I step back? If I do, there will be nothing there." I often see these *highly intellectualized manager parts and perfectionistic parts as staunch protectors of neglect*.

As you review your caseload, evaluate how many of your clients with neglect histories have strong storytelling, obsessive, or intellectualized parts. I see them as heroes working around the clock to ward off a wound they can't see, feel, or touch. I have found that working from the bottom up—focusing first on the body, then the emotions, and last the thoughts—can be an entry point to help some of these parts begin to separate.

Parts that protect by discussing, figuring out, and understanding often think they are the Self. But speaking from personal experience, it's relatively easy to differentiate them from Self-energy. That's because I can get bored, distracted, and disinterested in these parts as they ramble on about the details of their narrative. It's also common to feel ineffective with these parts after several unsuccessful attempts to help them separate and allow the Self to emerge. Several of my long-term clients are those who have wounds rooted in neglect. They stay in connection with me for years, with several parts of them clinging to an external connection because they're void of an internal one. These clients have a difficult time accessing Self-energy and struggle to complete a full unburdening.

Clients with neglect-driven systems have a *difficult time connecting with what they feel and struggle with making decisions*. Under normal circumstances, we use a combination of our thoughts, feelings, and intuitive sense to make a decision (i.e., a combination of our parts and the Self): "This makes sense to me. It feels right. I'm going to do it." Clients who have highly active thinking parts will obsess, contemplate, and talk about the pros and cons of a situation without ever making a decision. Thinking keeps everything in the head, making it harder to know what feels right. For example, one may think: "There are so many houses I've looked at that I like. I can't figure out which one is the right home to purchase."

When one tries to decide without self-connection, there's often *a sense of loss* associated with whatever is chosen. For instance: "If I buy the house on Green Street, then I can't have that beautiful bungalow on Laramie Avenue." These clients frequently ruminate, worry, get stuck, and can be misdiagnosed with OCD. However, if you begin to listen, you'll realize that it's their thinking parts working overtime to protect them from feeling the emptiness, loneliness, and loss that's buried deep down inside.

Consider the following dialogue between me and my client Brian:

"Brian, I've noticed that you're spending a lot of time in this session trying to figure out how many courses you should teach this semester," I said. "Do you have a sense as to what feels right for you?"

"There are too many good options," Brian responded. "I guess that's a good problem to have, but I know that the ones I choose will affect my supervisor's view of me. I just can't make a decision."

“Which would make you the happiest?”

“I have no idea.”

“Let’s take a moment and see if we can approach this from a slightly different angle,” I suggested.

“What if you stop trying to figure out which courses to pick? What shows up inside?”

“Nothing, nothing at all.”

“What’s happening in your body?” I asked.

“I feel a knot in my stomach.”

“Can you focus on that knot for a moment?”

“It’s like a pit in the bottom of my stomach. It makes me so nervous.”

“See if you can stay with it for a few seconds longer.”

“I don’t know what it is, but at least I’m feeling something for a change,” Brian commented after a few silent moments.

“Let the part that’s trying to decide and the sensations in the pit of your stomach know that we’re going to move slowly here. There’s no hurry at all; we’re just going to be with them right now in this moment.”

“That feels really good inside. You’re staying with me and helping me sort this all out. I didn’t get much of that as a kid,” noted Brian.

Here, the client begins to see that the importance is not so much about the correct decision but more about someone being there with him to help him sort out his feelings, thoughts, and bodily sensations. He had to do this all by himself growing up, and his thinking part did its absolute best to try and sort things out for him.

Another thing I’ve noticed about clients who suffer from histories of neglect is *my tendency to overshare*. Several of my clients present as if they haven’t a clue how to navigate their life, saying things like: “What do you mean? I don’t know how to do that. I’ve never thought about that before. What do you think I should do?” Neglect leaves clients at a loss, as if they don’t have any life experiences from which to draw, which may be true. Yet I’ve noticed that I have this penchant to share my personal experiences with these clients, and they tend to find it helpful in some way. It gives them something *real* to connect with. I might remark: “Here’s what happened to me when my oldest started dating” or “When I get frustrated with my husband, I sometimes feel this way.” Of course, we need to be careful about boundaries and what we’re sharing from our personal lives. Is it coming from the Self or from a caretaking part? But for these clients, I find that sharing some of my experiences helps them connect in a way that their neglected parts have not experienced before. I always ask myself: “What is the purpose of sharing? Whom does it benefit?”

Neglect is a silent injury associated with profound, long-lasting effects that can take years to fully resolve.

RESILIENCE

We know from the ACE study that there are significant developmental and mental health differences between children who have minimal trauma and secure early attachment experiences compared to kids with severe trauma and less secure early relational experiences (van der Kolk, 2014). There are, however, certain individuals who seem to experience horrific trauma and can somehow overcome it. They have something extra within them: the internal desire, drive, and capacity to overcome difficult circumstances, even when the odds are stacked against them. Who are these people? What do they possess? And how did they get this way? Is it nature (temperament) or nurture (environment) at work? Bessel van der Kolk believes that *resilience*, or the capacity to bounce back from adversity, connects to the level of security established with the primary caregiver during the first two years of life (van der Kolk, 2014).

I agree with van der Kolk, to some degree. I'm not totally sure what has contributed to my capacity to overcome my trauma as well as I have. It's true that I have a significant history of trauma, but I've also had an internal fighter in me most of my life. I remember as a child saying to myself: "I'm not going to let this get to me. I'll never stop fighting. They're not going to win." During my psychiatry residency years, I remember reading the book *Soul Murder* by Leonard Shengold and immediately thinking: "No, that didn't happen to me." I had this voice deep down inside of me that was never destroyed or silenced. Is this the Self? Is this due to secure attachment? Is this resilience? How did I get it? Why do some of us seem to have more of this than others? I've benefitted greatly from therapy and suspect I will forever continue my healing journey. I have an internal drive to improve and better myself. Whatever that quality is, I've seen it in some of my most severely traumatized clients. I'm grateful that I possess some of it too. It has saved my life.

POLARIZATIONS IN TRAUMA

Polarizations are extremely common with trauma. On the surface, it looks as though they perpetuate untenable circumstances, create immobility with no obvious solution, and result in confusion and frustration for all parts involved. Clients may say: "I like this guy, but he frequently pushes me too far sexually" or "I get how being suicidal protects me, but it causes so many problems with my wife and kids." When different parts hold seemingly different views, it's not only challenging for them, but it

also perpetuates tension and distress in our clients. When therapists approach the dilemma from fix-it or problem-solving parts of themselves, it can create frustration and feelings of inadequacy within the therapist too.

Polarizations rooted in relational betrayal are typically more powerful and extreme and create an all-or-nothing scenario in clients. This is how many trauma survivors frequently get diagnosed with borderline personality disorder, often due to unsolvable polarities within volatile relationships. You may hear clients reveal things like: “I love my dad, but he becomes a total monster when he drinks” or “I can’t live without my boyfriend, but I hate when he’s violent and disrespectful.” Even though each side of a polarization appears to hold important and opposing views, they often (but not always) protect the same wound. For example, a comment such as “I can’t live without my boyfriend, but I hate when he’s violent and disrespectful” most likely reveals two parts that are protecting a young attachment wound that wanted love and connection from an abusive caregiver.

Ponder the case of my client Charlie. He was able to truly listen to his numbing part. He learned how vital this part was to his system and how it protected him from the terror he experienced at the hands of his psychotic mother during childhood. Charlie was also able to separate and hear from the part of him that wanted to be more present and active in his life. Here’s a conversation we engaged in during a session:

“These parts have been at war with each other for years,” Charlie said. “I get it in a different way now—they both have legitimate arguments.”

“Charlie, can you check and see if they’re both protecting the same little boy inside who was hurt by your mom?” I asked.

After a few moments, Charlie responded: “Believe it or not, they aren’t protecting the same kid in there. The numbing part is definitely related to my mother. And the other part, the one that wants a fuller life, is connected to a part of me that was always on the outside looking in. You know—the last one picked for sports, never being invited to any of the school birthday parties, the last one asked to dance at prom.”

“I hear that, Charlie. What would you like to say to these parts right now, if anything?”

“I want to let them know they don’t have to fight anymore, that my heart is open to both of them, and I have enough love to go around.”

“Can they take that in from you, Charlie, and are they willing to give us some space to be able to help who they have been guarding for so long?”

“What I can tell you is this: There’s so much relief inside right now, and this is the first time a solution even seems remotely possible for each of them. This has been such a huge battle inside of me for so many years,” Charlie replied, his body now in a more relaxed and peaceful pose.

Self-energy is a tangible solution to many, if not all, polarizations. When it's available in both the therapist and the client, and they're open to hear both sides of the story, it usually becomes clear to all involved that each side has a valid point. It's also clear that everyone benefits, both inside and out, when they listen to suggestions from the Self.

Any time Self-energy is present, even if another part of the system is polarized with it, I have my client send that healing energy directly to the target part before we address the opposing part's concern. To demonstrate, consider the following discourse between me and a client named Linda:

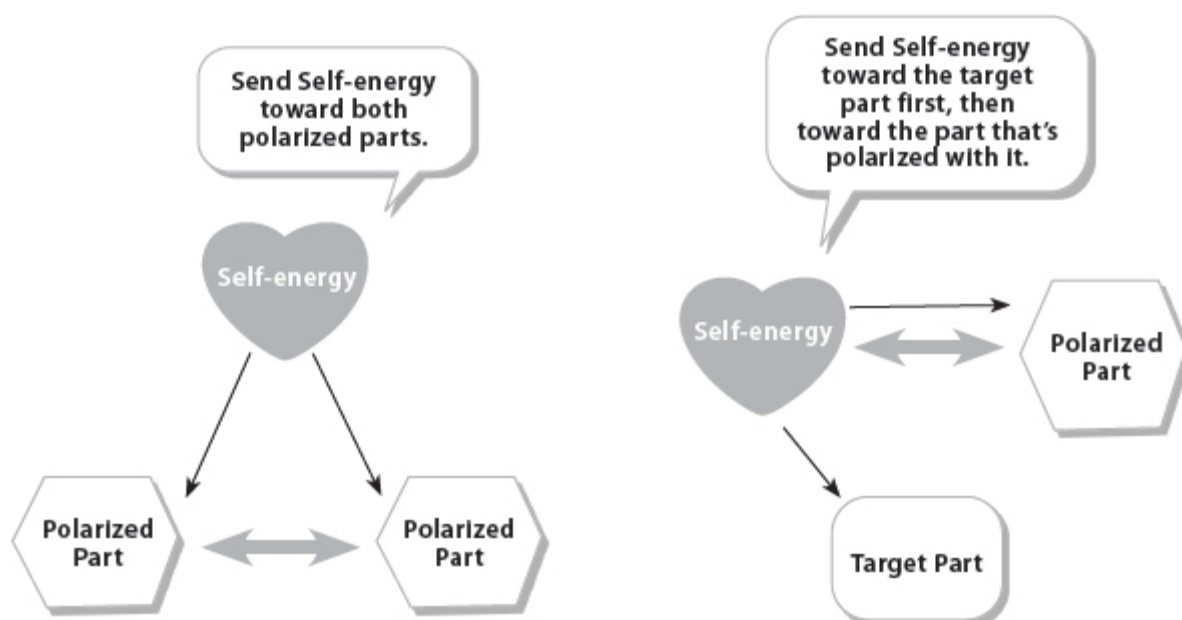
"I'm feeling mixed right now," Linda said. "I truly appreciate the angry part and how it's trying to help me. I'm also aware that there's a part of me who's afraid of the anger and its destructive potential."

"Linda, can you send those appreciative feelings toward the angry part first? Then see if it's able to take that in [sending Self to the target part]. Now let's get to know the part of you that's afraid of the anger and see what it wants to share with us [addressing the part that's polarized with the Self]. I'm sure it has a good reason for feeling this way," I said.

(Remember: Polarizations that involve Self-energy will resolve more quickly when the target part, in this case the anger, can take in the healing power of the Self.)

"The fearful part seems more relaxed now," Linda said. "She saw how kind and respectful I was with the anger. She really likes that."

When there's a polarization between two different parts, having the Self listen and validate both views can help resolve the dilemma. When the Self is part of the polarization, have the Self send positive energy toward the target part first. That way, the part that's polarized with the Self can witness and take in the benefits of the Self firsthand, which typically settles the conflict more swiftly (as shown in the graphic on [page 131](#)).



Polarizations tend to be an overcorrection to an overwhelming experience, but they are unfortunately the norm for clients who have been relationally violated. Conflicts, opposing views, untenable situations without a solution, and being stuck are commonplace. Protector polarizations are a brilliant and effective way to keep well-intentioned therapists confused and guessing while ultimately keeping the vulnerable, wounded feelings at a safe distance.

CHAPTER 19

Trauma, Feelings, and Loss

A NONTRADITIONAL APPROACH TO PARTS

Some clients don't take to parts language easily. Some have a hard time going inside, while others have parts that don't work in the typical IFS fashion. I find this common, but not exclusive, with clients who have highly intellectualized systems. These parts need to be in control. They need to talk extensively and for long periods of time before they'll relax and give permission. With these clients, I tend to employ IFS therapy without ever using any of the typical language. I don't say "part." I never ask anyone to "step back." And I don't ask my clients to "go inside." Instead, I engage with the client in a traditional talk therapy approach because that's what their system seems to require.

However, I always follow the IFS model in the back of my mind. As I talk with a client, I notice protective parts show up, and I listen for the wounds they protect. I engage parts in conversations without them explicitly knowing that's what we're doing, and I seek permission to access wounds. We're able to release painful experiences without formally going through the steps of the unburdening process. The therapy moves forward but at a much slower pace. Protectors eventually give permission, and wounds reveal themselves and release what they're carrying. All of this occurs without ever looking or sounding like typical IFS therapy.

For proof as to how this can work, take a closer look at my client Debra—an intelligent, high-powered business executive who runs several companies. Debra likes to be in control, and parts of her need to be in control. Here's a snippet of an exchange with Debra during a session:

"Can we pick up where we left off last week? I've been thinking about what you said about my relationship with my daughter," Debra said. "I agree with you that I'm afraid to talk to her."

"What do you think is your biggest concern?" I asked.

"When it comes right down to it, I think I'm afraid to be vulnerable with her. I guess I know I am. If I'm going to be honest with her about the affair, I worry that she'll judge me. I also worry that her drinking problems are related to her watching me and her mom fighting all the time."

"What if you were able to go and visit her and listen to her perspective first, before telling her about yours?"

"I think that's a great idea," replied Debra.

“Do you have any idea where your difficulty with vulnerability came from?” I asked her.

“My best guess is that it’s connected to my relationship with my mom and how she constantly criticized me whenever I did something below her high standards. But I’m not sure.”

“That makes sense to me, Debra. See what your gut is telling you,” I suggested.

“It feels right,” Debra said.

Debra was ultimately able to talk to her daughter, to listen to her perspective, and to be honest with her about the affair once she was able to identify her own fears and connect with her painful experiences related to the relationship with her mom. But notice that this exchange never used words such as “parts,” “stepping back,” or “How do you feel toward that part?” Yet you can see that I was still talking to her about her parts, learning about their concerns, helping them to unblend, and revealing the exile underneath her protectors.

TRAUMATIC AFFECT AND EMOTION

It’s important to be able to distinguish between feelings and traumatic affect. They’re not the same and are often inaccurately equated. *Traumatic affect is not an emotion but an intense expression of an overwhelming symptom or reaction that is typically held in protective parts.* It’s an exaggerated response to devastating or violating circumstances. Yelling, cutting, and being suicidal, hypervigilant, dissociative, or reactive are physiological reactions to overwhelming events, not feelings. Trauma also includes *feelings*, which can be held within protective parts (e.g., feeling angry, afraid, or remorseful) or exiles (e.g., feeling alone, unlovable, or worthless).

Both the reactions to trauma and the feelings associated with it can be intense. IFS has a way of directly dealing with these overwhelming responses. IFS therapists address protectors and exiles alike, asking them through direct access to *not overwhelm the system* so the Self can safely be present and compassionate with whatever they’re holding and wanting to share.

Throughout my personal therapy, I frequently confused traumatic affect with feelings. It was not until my trauma symptoms diminished and my protectors began to trust my Self more and stopped taking the lead that I was able to be with my feelings (my exiles’ emotions) more authentically. My feelings were more manageable when they were in sync with my Self. They began to make sense and feel aligned with my truth for the first time in my life. Traumatic affect is associated with reactivity, intensity, and impulsivity, and the energy it carries can be big but different from a feeling. Differentiating between traumatic affect (mostly protective parts) and emotion (mostly exiles’ feelings) helps pave the way toward releasing the painful experience once and for all.

LOSS, BETRAYAL, SAYING NO, AND LETTING GO

While loss is a normal part of life, it is difficult to manage. When we lose a family member, friend, or loved one, we experience an enormous amount of pain and grief. Complex trauma survivors, on the other hand, will often experience *any amount of loss in traumatic proportions*. When a loss occurs for them, not only are they experiencing emotions related to their current-day experience, but it also triggers repeated relational losses from their past. *It's a double trauma for them*. The result is additive, and the loss is experienced in catastrophic proportions. Survivors live through the loss of the present while reliving, yet again, losses from the past.

I feel like I've developed a subspecialty as a grief counselor because of my work with relational trauma survivors. For these clients, small losses can feel overwhelming, and more significant losses can cause them to completely decompensate. This awareness will hopefully help you remain in Self-energy when counseling a client who appears to be “overreacting” to what may appear to be minimal but which is experienced as profound when it's confounded by the past.

Betrayal also triggers a similar seemingly exaggerated response. When intimacy is violated, it's certainly hard to recover from. But with complex trauma clients, a betrayal often feels cataclysmic. I have rarely seen a trauma survivor recover, for example, from a partner's infidelity. It's not impossible, but it's extremely difficult due to the activation of young, wounded parts inside that still hold unbearable pain around the repeated relational rupture they endured years ago at the hands of their abuser.

Loss is particularly hard when it's associated with repeated relational disconnection. Take the case of an adolescent client named Abby, for instance. When Abby's mom is triggered, she loses her safe and reliable mother and is forced to interact with a reactive or out-of-control part of her. When her dad is drunk, Abby loses the father she loves and tries her best to please and placate his drunk counterpart.

These repeated relational losses create protective parts within our clients that have the exhausting job of doing whatever they can to prevent and protect against yet another painful loss. Unfortunately, this attempt by protective parts to seek out relationships that prevent reoccurring losses in childhood often results in clients choosing partners who will likely leave or abandon them.

I've become more aware over the years that many of the decisions I've made in my life have been rooted in loss prevention. For example, I have had *a hard time saying no to things*. When someone would make a request, I previously tended to say: “Sure, I'll teach next month,” “That would be fun—I'd love to be a part of that project,” or “Okay, son, you can buy a new bike.” When I went inside

myself to examine why it was so hard for me to say no, I heard: “If we say no, we’ll lose out on something fun and exciting,” and “If we say no, they won’t like us anymore.” Both of these parts of me would often take over and say yes to prevent me from feeling a loss of any kind, including the loss of a fun experience or the loss of a connection. I’m happy to say that I’ve been able to unburden these wounds, and my life is in much better balance now—thanks to IFS.

Are you aware of parts of you that react and respond so that you won’t have to experience the feeling of loss?

When we’re united with our truth in feeling, we experience a sense of well-being and know that we’re on the right path. However, being aligned can also involve some loss. You might think: “If I say yes to this, it means I’ll have to say no to that.” This is generally well tolerated, especially when it’s connected to a decision that feels right and is connected to the Self. If we say no to something that’s affiliated with what we *think* we should do, or what others want us to do, or what our protective parts want in the service of protecting a wound, we’re more externally driven, and the feelings of loss are generally more intense and harder to tolerate.

Saying no also requires *tolerating feelings in other people* too, which can be just as challenging. You might find yourself thinking: “If I say no to going out to dinner with friends, or if I say no to taking on a new client, I’ll have to tolerate them potentially being disappointed with me.” Worrying that others will respond in an angry or disappointed way is not an easy thing for trauma survivors, as it potentially activates the energy of their protective parts. Making decisions requires the capacity to tolerate missed opportunities, feelings of loss that arise, and uncomfortable feelings in others. When we connect to our feelings from the Self, as opposed to from parts, decisions are easier to make, and the feelings that emerge are easier to tolerate.

Struggling with loss can also result in *hoarding behavior*. I’ve worked with several clients over the years who’ve struggled with attachment wounds connected to an abusive or neglectful parent. They’ve developed protective parts that feel the need to hold on to everything and anything in a desperate attempt to avoid the pain associated with the loss of their primary attachment relationship growing up.

One client of mine traveled from one apartment to the next, always taking with her 15 boxes of stuffed animals that her father gave her when she was younger. I’ve joked with other hoarding clients about my making a home visit someday, awaiting the mortified look on their face because they fear that I am serious and would actually see what they’ve accumulated over the years. Sadly, many never feel able to move out of their homes because of the enormous amount of time and energy it would take to pack up all their belongings. I try to help these clients connect with what they need to hold on

to and what they can let go of. This is related to the items they've accrued, as well as their memories of people who have both loved and hurt them so much.

The biggest loss of all for complex trauma survivors is *the loss of their Self*. Miguel's wounded part said it perfectly: "As if it wasn't bad enough that my brother sexually abused me, that my mother was constantly depressed in bed, and that my dad was a raging maniac every time he got angry, through it all, I lost my Self over and over and over again." Repairing the injury created between parts and the Self as a result of trauma is one of the true gifts of IFS.

The following is a summary of parts commonly associated with loss.

- **Parts affected by betrayal and infidelity**
- **Parts that lose when someone is triggered or abusive**
- **Parts that don't say no in fear of loss**
- **Parts that agree or say yes to prevent loss**
- **Parts that hoard objects in lieu of people**
- **Parts that lose the Self as a result of trauma**

CHAPTER 20

Therapist Parts

From my perspective, the main focuses of IFS Level 1 training are to help therapists: (1) learn to be proficient at the model, (2) get to know their parts better, and (3) be more aware of and less susceptible to being triggered when counseling clients. When the therapist's parts get triggered by the client's needy, desperate, anxious, or avoidant parts, it can be challenging to actively listen to clients, respond appropriately to their needs, set limits, and maintain healthy boundaries. When we respond from a part of us, it has a particular energy associated with it that clients subconsciously perceive and react to from one of their parts. For example, when one of our controlling parts attempts to set a boundary, the clients can respond in several ways—from getting angry, pushing back, or disconnecting to appearing needy or desperate for contact and connection.

When therapists react from a caretaking part, they give too much of themselves in an attempt to rescue and help. This response might initially feel wonderful to some parts of our clients, but other parts of them become alarmed and feel like the therapist has crossed the therapeutic line, causing them to fight back or shut down in an attempt at self-preservation. When activated, *therapist parts often give too much, withhold too much, offer advice, or act rigidly*, which can trigger a range of protective parts in our clients.

I once had a client named Joe, for whom I was serving as a backup therapist. Joe emailed me with concerns about Sienna, his primary therapist, writing:

“Frank, can you please help with Sienna? She’s in the hospital right now for something related to her colon. Sienna keeps calling me at night before she goes to bed to make sure that I’m safe and not drinking or cutting. Can you please help me out here and tell her to stop calling me? She’s a therapist—she should be taking care of herself, not worrying about me.”

I realized immediately that Sienna's caretaker part had taken over, and I could only imagine what Joe's parts must have said or done to elicit that overprotective response in her. I knew that Joe could certainly get into some serious self-destructive patterns, so I refrained from criticizing or blaming Sienna. Instead, I had compassion for Sienna and her parts that felt overly concerned for him. I also

felt bad for Joe's parts that were experiencing Sienna as intrusive. Today, I no longer think in terms of countertransference, as I was taught in my residency training; I think about therapist parts in reaction to and in relationship with clients' parts.

Setting boundaries from the Self instead of parts feels quite different. We're calmer and clearer and speak from a different internal place, and our clients respond to our energy from a different place within themselves. They can feel the difference between our centered presence versus our driven agenda. When we're in Self-energy, they may think: "Thank goodness someone is in charge here."

I recently had to tell a client named Mary: "I'll do my best to be there for you whenever you need me. But as you know, I have two kids and travel a lot. There could be times when I won't be available for you, unfortunately. But if it's urgent and you haven't heard back from me, by all means, please try me again." I said these words from the heart, from my Self. Mary's response was positive: "Thanks, Dr. Anderson. That means a lot to me. I appreciate it." As a result, I rarely get calls from my clients during off-hours. They know I genuinely will be there for them when I can, and I know that they sincerely don't want to be intrusive in my life. This is what I call establishing *Self-to-Self boundaries*.

For clients who struggle with attachment trauma, the therapist's Self is a vital presence, especially during the early stages of treatment. Often, these clients do not yet have a lot of access to their Self-energy, and they rely heavily on the therapist's calming effect. I tend to call this the *Self of the therapeutic system*, or the collective Self-energy of the therapist and client. In some instances, the ratio could be wildly disproportionate—say, 98 percent therapist's Self and 2 percent client's Self—especially in the beginning of treatment. However, as clients begin to listen to their parts and learn about the parts' jobs and fears, more Self-energy becomes available to them. Eventually, the ratio becomes a 50-50 share of the collective Self-energy between therapist and client. Soon, the client's Self can lead their internal system more independently.

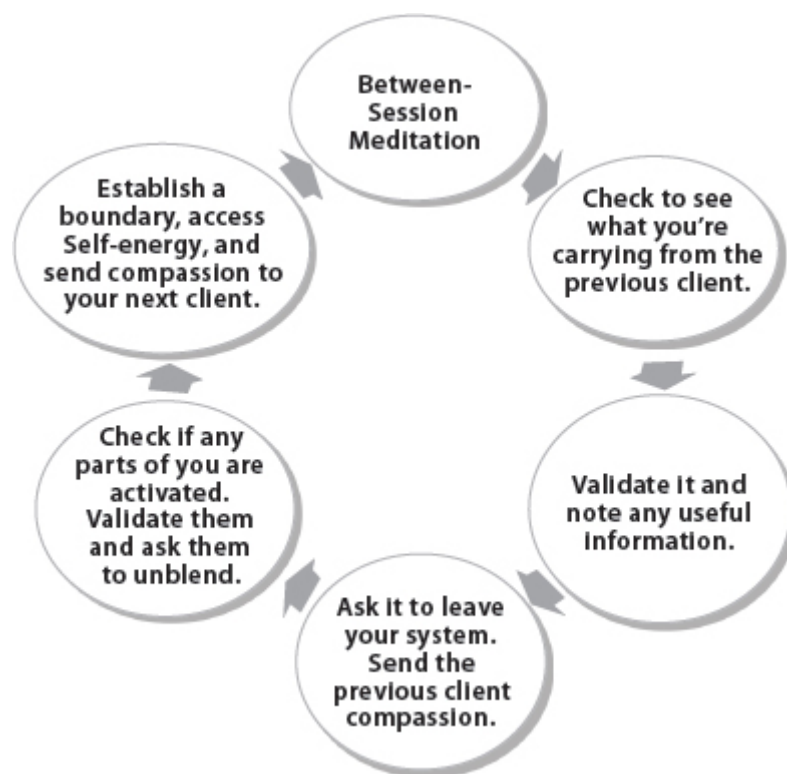
When the client's Self takes the lead—a process Dick Schwartz calls *passing the baton*—it can be particularly challenging for some of the client's young attachment parts. Remember that these young parts have developed a strong trusting relationship with the therapist's Self, and they're often terrified of losing that connection when the client's Self begins to take the lead more often. From the young parts' perspective, the client's Self was the one who abandoned them at the time of the trauma, not the therapist's Self, so they are often not thrilled about this transition.

THERAPIST SELF-CARE: BETWEEN-SESSION MEDITATION

I now make it a general practice to eat a few almonds, visit the washroom, and engage in a brief meditation between sessions. I take a moment to go inside, scan what's there, and identify any

feelings or energy I've picked up or carried from my previous client. I then acknowledge the presence of these unwanted elements and ask them to leave my system. I often gain important insights about my clients during these quiet moments of introspection following a session, and I will jot down these observations in the client's chart.

Once I've cleared the energy from my previous client, I'll internally send them love and compassion and check to see if any of my parts became activated. If so, I listen carefully to these parts, validate their feelings, energetically reestablish my boundaries, and ask these parts to relax and allow Self-energy to return within me. Once I sense this shift, I open up space for my next client, internally sending them love and compassion before I externally summon them from the waiting room so that we can begin our session.



ATTACHMENT AND THE THERAPIST

Clients with attachment trauma have a range of parts, and among them are exiled parts that can develop strong attachments to their therapist. With the natural ebb and flow of the therapeutic process, these parts of your clients can react in not-so-obvious ways to your comings and goings.

During my residency, where most of my supervisors were psychoanalysts, we spent an inordinate amount of time focusing on the transference issues that got stirred in our clients when we took time off or went on vacations. It felt much more theoretical than actual to me back then. Our clinical rotations lasted from three to six months at the most, and I assume it was possible, although unlikely,

that my clients got that attached to me in such a short timeframe. I felt like the supervisors were unduly emphasizing my importance in my clients' lives. Looking back, I was probably right.

Fast-forward to now, with me being in full-time private practice for several years. I acknowledge that many of my clients with attachment trauma definitely seem to be affected by my comings and goings—only not in the way my supervisors said it would unfold. I find that my clients often react in disguised ways that can take me time to identify. One client named Calvin would casually bring up his desire to try EMDR with another practitioner while I was away. Being an open-minded therapist, I explored with Calvin the pros and cons of this decision. It took us both time to figure out that his desire to experiment with EMDR was connected to my increased travel schedule. Complicating matters was the fact that Calvin was also a therapist. His exiles were, for the most part, well-hidden. When we would explore a possible connection to his interest in EMDR and my upcoming departure, Calvin would experience tension in the chest, tightness in the shoulders, or stomach pain. It took us over a year to connect the activation in his body with his exiled part, who held grief, despair, and occasional suicidal ideation due to repeatedly being dropped off at boarding school as a child.

Another client, Wanda, had parts that were insistent about how useless, unsupportive, and unavailable her Self was. It took us a while to associate this feeling with my periodic leaves from practice. After several years of therapy, she was eventually able to connect with and speak for the parts of her that felt abandoned by me—parts that, in her words, were “constantly being thrown to the wolves.” We quickly learned that they were referring to Wanda's Self as the wolves.

Again, making these connections around loss can be challenging, especially with a client who has parts that are young, ill-defined, and largely unknown to their system. It's not as obvious and direct as my supervisors had explained it would be during my residency. But the importance we carry for the young parts of our clients who have suffered from attachment trauma is certainly worth paying attention to.

BEING RELATIONAL BUT NOT TAKING IT PERSONALLY

Being connected and relationally available to your client while maintaining appropriate distance, keeping proper perspective, and remaining in Self-energy can be challenging, especially when you're dealing with desperate or dysregulated parts of the client. I've found it difficult at times to not get taken over by certain parts of me that developed in my childhood. When I was younger, my parents would frequently tell me what I was feeling and what I should be thinking. I remember my response

was more to challenge than comply, saying things like “No, that’s not what I’m feeling” or “Don’t tell me what I’m thinking.” I had a stubborn part that I later learned was fighting for self-preservation. How grateful I am for that part!

However, today when my clients tell me what I’m thinking or feeling, my younger part still tends to show up. I have always admired Dick Schwartz and his ability to not internalize what clients project onto him. Triggered parts of clients will often cast their beliefs about their abuser onto their therapist, particularly when they subordinate or sense an unbalanced power differential within the therapeutic relationship.

When clients bring these perspectives and distortions into the office, *I have found it important to validate the part’s perception first* (as it’s coming from an important place in their past) while trying my best to not take it personally and remain relationally curious, available, and connected. This is easier said than done. Remember what Louis Cozolino said: that most of the input to the cortex comes from internal processes, and we scan for examples that prove our preexisting beliefs, primarily driven by fear to avoid danger (Cozolino, 2010). I find this insight extremely helpful. I try to remember that a client’s perception of me in those moments is often rooted in their past experience and has very little, if anything, to do with me.

I don’t always do such a great job with this struggle because my associated wound is not fully healed yet. I am still sometimes affected by the relational aspects of a connection to a client. Case in point: Consider my experience with a client named Peter. Peter’s standing appointment time had changed from noon to 11 a.m., and the first week this change went into effect, he was late. It was approaching 11:30 a.m., so I sent Peter an email reminding him of the time change. I didn’t hear from him until the following day. He called and left me a phone message in a spiteful tone, saying: “I’m so upset that I missed my appointment, I guess I’ll see you next week.” The following week, Peter showed up 15 minutes late for his appointment and was furious at me. Here’s an exchange we had:

“I’ve been waiting for your car to show up in the parking lot,” Peter said. “I didn’t think you were here. Why didn’t you call me and tell me you were here?”

“I’m sorry, Peter. My car’s in the shop right now. I drove a loaner vehicle today. I’ve been here the whole time waiting for you,” I replied.

Peter began to escalate. “You know, you could have called last week instead of emailing me, and you should have called me today when you noticed I was late,” he said angrily.

If the parts from my childhood weren’t activated, I could have validated his experience and said something like:

“Peter, I see that you’re angry with me for not reaching out to you by phone last week, and you wish I would have called to see where you were this week. You seem pretty upset. I totally get it. I’m interested in hearing more about your experience.”

Then, hopefully, after his angry part felt seen and heard, we could have explored what was behind this protector’s response. Instead, I replied this way:

“Wow, Peter. It’s really hard for me to be attacked like this. I did reach out to you last week, in the way I do with all of my clients, and I am sorry that you didn’t know that I was driving a different car this week. But it doesn’t feel great to be attacked like this. Hopefully, we can turn this into an opportunity to help you learn how to work through difficult moments in relationships. I know this is something you’ve struggled with in the past.”

I observed Peter tear up for a brief moment, and then his angry tone and face quickly returned. I suddenly realized I had lost an opportunity to genuinely connect with Peter and his vulnerable parts by sharing my experience with him before I was able to listen to and validate his parts.

I don’t think we do our clients any favors by allowing them to *be in their parts* and barrage us with their hurtful attacks. On the other hand, challenging their projected beliefs can further alienate and enrage their extreme parts. The beauty of IFS is that it doesn’t do either of the above but incorporates both. The Self can tolerate intense feelings and distorted views, avoid taking them personally, and name what’s happening in a non-shaming way—all while still maintaining the open curiosity to access the true origins of the trigger.

WHAT ARE WE OFFERING?

As a consummate caretaker, I have grown to value and appreciate what I consider to be IFS’s unique perspective on healing, which can be summarized in the answer to an important question: What are we offering our clients? Are we offering a healthy corrective relational experience that is therapeutic, or are we offering to help them heal their wounds by connecting to their Self?

IFS offers the latter. We help our clients access their Self-energy, and we support their Self as the corrective, healing experience for their parts that carry past pain. In my experience, as echoed by one of my clients, the Self can offer what no one else can: the ability to repair the breach that formed during the original assault, as well as a unique bonding experience powered by true acceptance and love. In IFS, therapists can offer their Self as an adjunct to healing, and when they do, ideally it

comes from a place of compassion, not empathy. Remember that compassion is a desire to help that includes care, distance, and perspective (Self-energy), while empathy is an activated part of the therapist that resonates with the client's painful experience.

Some therapeutic modalities hold a different view. They posit that the therapeutic relationship is corrective and healing for clients. I'm not saying they are wrong and IFS is right. I'm simply pointing out a difference between IFS and some other models of therapy. *In IFS, the internal Self-to-part connection is the primary healing relationship, and the therapeutic relationship is adjunctive or secondary to that.* In other words, the internal relationship is reparative, and the external relationship is supportive of that process.

Consider the case of my client Emma. During a particularly poignant moment in one of her therapy sessions, Emma's little girl part took over and said to me:

"Will you be patient with me? Will you not leave me?"

In the gentlest way possible, I said: "Of course, I'll be patient with you, but I can't promise you that I won't leave you." Emma looked up at me and started to cry.

"I'm totally committed to helping you," I continued, "but what if I were to tell you that Emma can be there for you in a way that no one else can or ever was? Would you be interested in that?"

"Don't worry. I'm fine on my own," Emma said in her self-reliant caretaker voice. "I always have been, and I always will be. Daddy left me, the maid left me, and mommy left me too. Everyone leaves, and I learned how to take care of myself."

"What if I were to show you there's an Emma in there you haven't met yet who is capable of being there for you and loving you in the way you deserve? Are you interested in that?"

"Perhaps," responded Emma's part.

Here, the offer is clear: Emma's Self can be the corrective healing agent that her wounded parts need and want. Often, younger parts aren't yet fully connected to the Self I'm offering them. That's because the chasm between the Self and the part bearing the betrayal has not been fully repaired yet, much like we discussed earlier between the target part and the current-day Self. *We also introduce the wounded part to the Self of today*, the Self that is capable of listening to the client, being there for her, loving her, and not abandoning her as before. The Self of today is older, wiser, and more capable than it was back then. This is a result of age, development, capacity, and time. Again, Self-energy is the primary offer in IFS; the therapeutic relationship, although extremely important, takes a back seat and serves an adjunctive or supportive role.

CHAPTER 21

Common Comorbidities

REAL MIND-BODY MEDICINE: THE INTERSECTION BETWEEN PARTS AND BIOLOGY

In IFS, we often identify areas of the body linked to parts. One of the first questions we ask our clients when we're trying to identify the target part is: "Where is the part located in or around your body?" As I've described earlier, I believe that parts live in the mind and utilize the brain and body to express themselves. This intersection between parts and the body is important, especially when it comes to medical illnesses and physical symptoms.

How much are your client's repetitive headaches the result of a part trying to get their attention, and to what degree are they related to cluster migraines, to which the client is genetically predisposed? *It turns out that parts are incredibly helpful at sorting out this question;* more often than not, it's a percentage of both. For example, 60 percent of the migraine may be connected to parts while 40 percent is biologically rooted.

I have had a long history of what is called "cough variant asthma." This means that whenever I get a cold, I'm susceptible to experiencing an asthmatic reaction in my lungs that causes a chronic cough that can last for months. This requires me to take several medications, including inhalers, antihistamines, and steroids. There are some seasons when I've gotten sick and it doesn't trigger my asthma, and other times when I experience repeated asthmatic reactions.

I have done a lot of work in therapy with my parts around my reactive airway disease, and I've learned a great deal from them about my propensity for this reaction in my body. They've told me my asthma is genetic to some degree. (I generally hear this information from what sounds like a collective voice in my head.) My father has an overactive immune system that manifests as rheumatoid arthritis; for me, it presents as a hyperimmune response in my lungs. I've also learned that parts will sometimes show up in my throat to remind me that I'm out of alignment in some important way and not speaking my truth.

I have conflict-avoidant parts too and have a hard time speaking up when there's something difficult I need to say to someone. It often takes me by surprise when I get this information from my parts because I wasn't conscious of it. But when I hear them speak up to me, it feels right.

For example, I knew in my gut it was time for me to leave the Foundation for Self Leadership (FSL). I had spent five wonderful years connecting with the IFS community, working with wonderful board members, learning about how to run a not-for-profit organization, and supporting the expansion of IFS. It was hard for parts of me to acknowledge that it was time to move on, and it was even harder to speak this truth out loud to my friends and colleagues. At the time, I was traveling a lot and caught a cold, which triggered a severe asthmatic reaction. It wasn't until I began taking prescribed steroids, which cause numerous unwanted side effects, that I started asking what, if anything, was going on from the parts' perspective. I heard: "You know in your heart it's time to leave, but you're afraid of disappointing everyone."

After hearing this, I was able to connect with and heal the younger part of me that was terrified to speak up to my father as a child. Doing this piece of internal work made it much easier for me to voice my intentions and tell those I loved and cared about that it was, in fact, time for me to leave. My parts watched as I told the FSL board about my departure, and they were surprised at how everyone reacted. No one yelled or raised dissent. Everyone was happy for me and supportive of my decision. And my cough, yet again, resolved.

Parts and biological conditions have a strong relationship with each other. As Dick Schwartz often says, *parts can push the biological button*. My parts were telling me: "We caused the asthma attack to get your attention because you are not speaking up." And the reverse is also true. *Biological conditions can often affect parts*, causing them to say: "This isn't us—we have nothing to do with the asthma attack. It's the winter, the heat is on, the air is dry, and you're allergic to dust." At other times, it's a combination of both parts and biology, where they share a percentage of the responsibility. All we have to do is ask; parts are usually willing to help sort things out.

I recently worked with a client named Antonio who was recovering from a heart attack. Here is a sample of the dialogue we shared:

"Antonio, I wonder if you're open to listening to your parts about your recent heart attack," I said.

"What do you mean? Do you think I caused it or something?"

"No, I'm not saying that at all. I think that there's a strong mind-body connection, and perhaps your parts might be able to help us understand if there's any connection between what they're carrying and your physical symptoms."

"I guess so," Antonio said.

"Just ask if any of your parts knows anything about your heart attack."

“This is fascinating,” Antonio said with his eyes closed. “I would have never guessed this in a million years. What I’m hearing is that there is a connection to my job: ‘You’ve worked so hard for so many years to become a partner at that firm. You gave them everything—long days and many nights and weekends away from the family. You busted your butt for them, and now this new management team is treating you like crap.’ What they are telling me is true. I feel like I have no power anymore. My parts are saying that they’re brokenhearted.”

“Wow, that makes so much sense to me, Antonio,” I responded. “Let them know that we’re hearing this.”

“Oh, you bet I will. I had no idea that this was going on inside,” Antonio said, who then paused a moment. “They say it’s 50-50. They say I’m not eating clean and not nearly exercising enough—that half of this is medical and the other 50 percent is about their heartbreak.”

“Let them know you’re really getting all of this, Antonio.”

“You know what? This is an echo of my childhood. I worked so hard as a kid too, trying to get my parents to see me and love me, and it never worked. They were always too busy with one thing or another. They never seemed to have the time to care about me. Now I feel like it’s happening all over again at work.”

“Let your parts know that there’s a way we can help them with the feelings they’re carrying about your childhood and that it most likely will help you better manage the current problems at work. Are they interested in that?”

“Yes, definitely. They’re so happy we asked them about all of this. They said I wasn’t caring about them, in the same way that my employer and parents didn’t care about me.”

“All of these layers and connections make a lot of sense, Antonio. What do you want to say to them about that?”

“I’m not going to repeat this pattern. The buck stops here,” he said with conviction.

When we hear from parts that the anxiety, difficulty sleeping, and worrying is related to them, we learn how the symptoms help to protect, and we gain access to healing the underlying wound. But when parts say the anxiety, difficulty sleeping, and worry is not theirs, we must consider whether it’s biologically based and perhaps associated with symptoms of depression, for example. *Typically, when symptoms are rooted in a biological condition, they affect the whole system, and all parts are equally distressed by them.* We then ask parts if they would like help with the anxiety, sleep trouble, and worry. If they all agree, we explore medical solutions to address the problem.

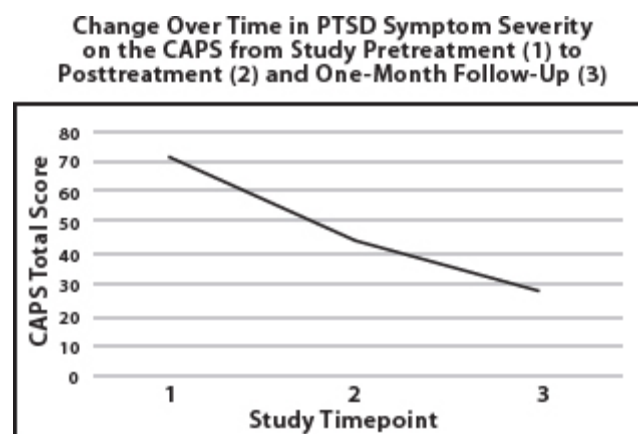
I believe that most medical problems and comorbid psychiatric conditions from which people suffer are partially due to their overall system being out of alignment in important ways. Holding on to unexpressed feelings, engaging in activities that aren’t aligned with our truth, or ignoring our inner

voice can manifest as either mental or physical symptoms. Parts can use our bodies to communicate with us that something inside isn't quite right.

Gabor Maté speaks of this phenomenon in stating: “Mind and the body are inseparable, and illness and health cannot be understood in isolation from the life histories, social context, and emotional patterns of human beings” (Maté, 2011, p. xi). Maté made the bold statement that several of his patients who suffer from a wide range of illnesses associated with chronic inflammation have never learned to say no in important areas of their life. He is also careful not to blame people for their illnesses: “There is no responsibility without awareness.” His goal is to promote learning and healing, not blame and shame, which he believes is overabundant in our culture. Maté advocates for self-awareness and self-connection in the service of disease prevention. He's a fan of IFS, and I'm a fan of his perspective on the connection between trauma, emotions, and medical illness.

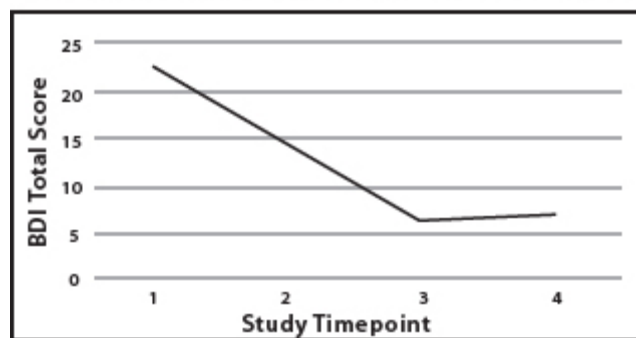
IFS TREATMENT FOR PTSD AND COMORBID DEPRESSION: A PILOT STUDY

I was fortunate enough to be involved in a research study supported by the FSL that examined 13 subjects with PTSD who had histories of multiple forms of childhood trauma, including sexual abuse (65 percent), psychological maltreatment (65 percent), and physical abuse (59 percent; Hodgdon et al., 2017). The subjects participated in sixteen weeks of IFS therapy, and after treatment, 12 of the 13 subjects (92 percent) no longer qualified for a PTSD diagnosis according to the Clinician-Administered PTSD Scale (CAPS; Weathers et al., 2018).



Statistically significant decreases in depression were also noted, as measured by the Beck Depression Inventory (BDI; Beck et al., 1961). Decreases were seen for affect dysregulation, dissociation, disrupted self-perception, interpersonal relationships, and systems of meaning as well.

Change Over Time in Depression Symptom Severity on the BDI from Study Pretreatment (1) to Mid-Treatment (2), Posttreatment (3) and One-Month Follow-Up (4)



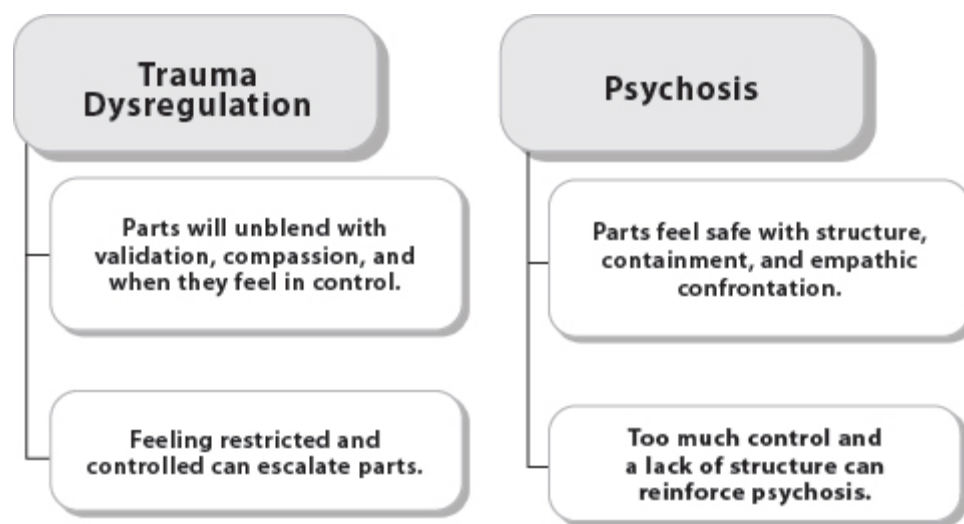
PSYCHOSIS, BIPOLAR DISORDER, AND TRAUMA

Years ago, I attended a residency program that primarily served clients who were homeless and had no health insurance. Many of them suffered from major mental illnesses and severe substance use. Because the majority of our supervisors were psychoanalysts, we learned to explore the function or underlying meaning of a client's psychotic symptoms while simultaneously treating the chemical abnormalities occurring in their brain. I discovered that if you feel alone or isolated, being paranoid can make sense. Believing the FBI or some covert operation is spying on you around the clock, or being convinced that Jesus talks to you regularly, can make you feel special and important if you're feeling inadequate, lonely, or insecure.

I learned long ago—as IFS teaches today—to view the positive intention of all symptoms, even with psychotic symptoms or parts that hold psychotic beliefs. I have never met a client with bipolar disorder who didn't describe what they were fleeing from after they'd recovered from their most current manic episode. The majority of the time, these clients had some underlying trauma history or traumatic loss that they were escaping during their manic periods.

In IFS, we also look for the positive intention of any psychotic thought process. And we try to sort out which portion of the symptom is affiliated with some form of protection and which portion is biologically based that could benefit from medical intervention.

People with severe complex trauma, PTSD, and dissociative disorders can *appear* psychotic, especially when they're dysregulated. The neurobiological abnormalities resulting from their trauma make it difficult to adequately process thoughts and feelings when they're triggered. They can present as confused, distressed, and dysregulated in a way that mimics a thought disorder. *One of the differences between psychosis and trauma dysregulation is that the latter recovers once the client is no longer triggered, but psychosis persists well beyond the present activation.* Parts also require different interventions to help them unblend.



Clients who suffer from trauma have rarely had their perception of reality validated, and their control has often been violated or taken away. Therefore, traumatized parts need to have their experiences validated and to feel a sense of control so they can unblend. However, with psychosis the opposite is true: excess validation, too much control, and reinforcement of their distorted beliefs and feelings can perpetuate or reinforce psychotic thinking. These thought-disordered parts need limits, structure, and a sense of safety, which can be accomplished when someone gently names what is actually happening in reality because their internal experience is often frightening, chaotic, distorted, and internally overwhelming.

Clients with severe attachment trauma or those who are labeled with disorganized attachment can have young exiled parts that develop distorted or psychotic attachments. These young parts often hold confusing experiences or carry mixed burdens from their past in connection with their primary caregivers. For example, this client may say: “My father is loving during the day and a violent monster at night” or “My mother is physically intrusive, but I still love her and need her for survival.” I’ve had two such clients in my career, both of whom I treated for several years. In each example, the clients formed psychotic attachments to me slowly over time, and it was hard for me to recognize it until it was profoundly entrenched.

I made the mistake of not confronting the distorted thinking early on in the treatment. In retrospect, I was too gentle and supportive of their view and unknowingly reinforced their beliefs. These young parts don’t have the cognitive capacity to differentiate reality from their perception of reality, and there was often no one there to help them do so. In addition, these parts will often feel blamed or shamed when the reality is pointed out to them. The therapist’s Self can conduct delicate conversations with these clients—conversations that name and hold the balance between reality and the exiled part’s experience. If both are not addressed or closely monitored, psychotic attachments can be fostered.

Working with parts that hold psychotic beliefs generally takes longer and is more challenging. *This is due to the delicate balance required when naming reality without shaming the part and validating the experience without fostering distortion.* It also takes time to decipher the language of psychosis. However, when healing occurs, clients feel tremendous relief and are enormously grateful because they no longer have to carry nonreality in the service of survival.

CHAPTER 22

Medications and Trauma

PARTS AND MEDICATION

Medications play a significant role in treating trauma. Most clients experience at least one comorbidity or a part overtaken by symptoms like depression, panic, obsessive worrying, attentional difficulty, or sleep problems. Therapists need to work closely with clients to determine if a medication trial is warranted. For a more detailed exploration of the relationship between medications and IFS, refer to the chapter I wrote, titled “Who’s Taking What? Connecting Neuroscience, Psychopharmacology, and Internal Family Systems for Trauma,” which appears in *Internal Family Systems Therapy: New Dimensions* (Anderson, 2013).

When therapy feels stuck and progress has been halted, ponder comorbid biological conditions and potential medication trials. A medication can serve as an effective *unblending agent* when clients are too stuck in their parts and have difficulty achieving separation. Medications tend to work more effectively when *all parts agree to take them*, and there are usually fewer side effects when all parts give consent.

Parts have been known to *interfere with medications* too, and they can “create” side effects when not adequately consulted. Parts know how to use the internet and scour for facts that suit their interests. They’re even capable of blocking medications from working. I’ve heard parts tell me: “I’m the one that blocked the Trazodone from making us sleep” and “I stopped the Zoloft from working.”

I once had a client named William who had a history involving early attachment trauma that included verbal abuse and neglect. He struggled with anxiety and dissociation, and his parts had a hard time separating from him so that he could get to know them better. William spent most of our time together being overwhelmed by anxiety and feeling numb, detached, and dissociated. Trying direct access proved helpful at times, but William’s physical symptoms dominated the sessions, preventing us from making progress.

I asked William to check inside and see if his parts were interested in taking any medications to help with his anxiety or dissociation. But it quickly became clear that taking medicine for anxiety

wasn't a possibility due to his dissociation. All of William's parts (except the dissociative one, which would not relinquish control), wanted help for the anxiety and were willing to try pharmaceuticals. Over time, the dissociative part hesitantly agreed, and William started on a low dose of Celexa.

After a few weeks, his anxiety lessened, and he was able to benefit from the space the medication provided for his system. It opened the door for him and me to engage the dissociative part more actively in a discussion. We learned that this part was the last responder and felt the need to jump in when things got bad enough inside. The dissociative part became active when William was 11 years old, when he started witnessing his mother suffer beatings by his father. We both appreciated the necessity of the protection that the dissociative part provided, and the part appreciated being known and respected for its role. It also appreciated not being forced into taking a medication. The dissociative part was eventually willing to soften further and would accept a low dose of Abilify, taken as needed when things got too intense for it. As you can see, working with parts around medication decisions often reveals clinically significant information.

Manager or preventive parts tend to *love* medications and are usually willing to take them, as opposed to extreme or reactive parts that need to be in control and typically refuse prescribed medications or will only take them as needed. I encourage all therapists to keep their own parts and personal views in check around medications and to stay closely connected with their clients' wishes throughout the prescribing process. Prescribers generally have minimal time to spend with clients, so it's important for therapists, who generally have a more trusting relationship with clients, to take the time and help them sort out their fears and to accept the risks of allowing a foreign substance into their bodies.

Wounded parts have also been known to request help from a medication, especially when they're struggling to manage the intensity of overwhelming traumatic affect. For clients with trauma histories, it's important to achieve harmony among all parts and to resolve opposing views before you refer them to a prescriber. Their success will greatly improve if all parts agree.

PSYCHEDELIC-ASSISTED PSYCHOTHERAPY

Psychedelic-assisted psychotherapy is a rapidly expanding treatment option for trauma survivors. Under the right circumstances, it can provide additional help and support for some who have struggled to heal from their overwhelming life experiences. The Multidisciplinary Association for Psychedelic Studies (MAPS) is an organization spearheading much of the research associated with the psychedelic-assisted psychotherapy movement. They have supported research connected to 3,4-Methylenedioxymethamphetamine (MDMA), the main psychedelic drug currently used in PTSD-

assisted psychotherapy. MDMA is also being studied for the treatment of anxiety associated with life-threatening illnesses and for social anxiety in adults with autism spectrum disorder.

MAPS also advocates clinical trials of medical marijuana for the treatment of PTSD among war veterans and ayahuasca-assisted treatment for PTSD and drug addiction. It has examined LSD-assisted psychotherapy for anxiety and has sponsored studies for ibogaine therapy for drug addiction. In addition, psilocybin, also known as magic mushrooms, is being studied for the treatment of major depression, alcohol dependence, OCD, and cancer, among other things.

Ketamine, the only FDA-approved psychedelic to date, is currently prescribed for treatment-resistant depression and is being studied for alcohol and heroin addiction. It is also being used in conjunction with psychotherapy to treat a variety of mental health-related issues. I am trained in ketamine assisted psychotherapy (KAP) and look forward to exploring the various ways it can help trauma survivors get beyond their extreme protective parts and heal their underlying wounds.

It's encouraging to see this group of medicines being used to treat a wide range of mental health-related issues, and I suspect their use will further expand as the research evolves. MDMA, the most well-studied psychedelic to date for treating PTSD, seems to be beneficial in helping clients increase self-compassion (Self-energy, in IFS terms). It has been shown to reduce defenses and fear of emotional injury (i.e., protective parts) and make unpleasant memories less disturbing while enhancing communication and capacity for introspection (Multidisciplinary Association for Psychedelic Studies, 2020). I support the use of MDMA for clients with extreme trauma histories who have a difficult time accessing Self-energy on their own.

However, not all psychedelic drugs should be treated equally because they each work on different receptors and neurotransmitter systems, and they have a different mechanism of action. For example, we may discover that MDMA is more beneficial for clients who have difficulty accessing Self-energy because it increases self-compassion. We may see that ketamine, which decreases excitability, is more helpful for protective parts that have trouble relaxing or stepping back.

As research and clinical experiences continue to expand, we'll get a better sense as to which medications and dosages are effective for which clinical circumstance. It's important to remember that these medications must be used with caution because they do have abuse potential and are harmful when used in excess or without proper supervision. The preliminary results look quite promising, and I truly believe that this group of medicines will provide innovative approaches and new exciting treatment options for a variety of medical and mental health-related issues. I encourage you to visit www.MAPS.org for further information.